THE LEGAL RESOURCES FOUNDATION (LRF)

versus

THE MINISTER OF HEALTH AND CHILD CARE

And

THE MINISTER OF WOMEN AFFAIRS. COMMUNITY, SMALL AND MEDIUM ENTERPRISES DEVELOPMENT

And

THE MINISTER OF PUBLIC SERVICE. LABOUR AND SOCIAL WELFARE

And

THE ZIMBABWE GENDER COMMISSION

And

THE ZIMBABWE HUMAN RIGHTS COMMISSION

HIGH COURT OF ZIMBABWE
CHIRAWU-MUGOMBA J
Harare, 9 March, 14, 15, 16, 20, 21 April 2026

C. DAMISO, for the applicant
T.S.SHANGWA, for the respondents

CHIRAWU-MUGOMBA J:

INTRODUCTION

A June 2023 report titled, 'National assessment of adolescent pregnancies in Zimbabwe' commissioned by the government of Zimbabwe in partnership with UNESCO and UNFPA these being United Nations agencies paints an alarming picture. Part of the issues assessed included the prevalence of adolescent pregnancies in Zimbabwe. Adolescents are described as any persons between the

ages of ten and nineteen years. Of an estimated 1, 706, 946 antenatal care bookings between the years 2019-2022 among women of child bearing aged ten years and above, twenty-one percent were adolescents. The rate of pregnancies amongst adolescents was twenty- three point seven. The age of 16 years was the mean age of first sex. It is against this background that the applicant has approached the court in terms of s85(1)(d) of the constitution as read with R107 of the High Court Rules. The relief sought by the applicant is a declaration of constitutional invalidity of s35(1) and (2)(b) of the Public Health Act [Chapter *15:17*], hereinafter ' the Act', and for consequential relief.

BACKGROUND

The applicant is a private voluntary organisation operating in Zimbabwe. Its objectives *inter alia* are to initiate and support projects which promote the development of legal resources in Zimbabwe and to also broadly promote activities that address access to justice. Through its director, one Valerie Zviuya, applicant avers that it has interacted with many young people regarding access to sexual and reproductive health rights, (SRHR) and has become convinced that s35(1) and (2) (b) of the Act imposes a significant hurdle to accessing reproductive health by sexually active adolescent girl. That this is a violation of their right to health care protected under s76 of the Constitution. These rights include access to reproductive health care services. This right embodies four basic elements of availability, accessibility, acceptability and quality. The application concerns access to SRHR by persons below the age of eighteen years. Section 35(1) states that informed consent refers to the provision of a specified health service given by a person with legal capacity to do so who has been informed as contemplated in s34. Section 35(2) provides that health services shall not be provided to a user without their informed consent except in the circumstances set out under paragraph (a) to (e). The reference to legal capacity means that persons below the ages of 18 lack the capacity to provide informed consent. As a result, they have no access to health services that they may require including SHRH. The requirement that these persons can only access the services through the informed consent of a third party constitutes an unjustifiable limitation of the right to health care as protected by s76.

This third party can be a parent or other guardian. The report alluded to in the introduction says it all. When adolescents engage in sexual activity, they do so clandestinely. Social norms and taboos mean that they are not free to discuss matters of SRHR with parents. The report notes the consequences that arise from the limitations imposed by the Act. Ultimately adolescents find it difficult to access SRHR services. Other related rights such as education and life are also negatively impacted. There is a further disproportionate impact on the girl child thus infringing the right to equality and non-discrimination. Accordingly the applicant seeks the following relief as enunciated in the draft order.

1. Section 35(2) (b) as read with section 35(1) of the Public Health Act [Chapter *15:17*], is hereby declared constitutionally invalid to the extent that it defines informed consent in a manner which precludes adolescents from providing their own consent to reproductive health services, such preclusion limiting their free access thereto and amounting to a violation of:-

    a. The right to health care protected under section 76 of the constitution; and

    b. The principle of the best interest of the child protected under section 81 of the constitution and is hereby set aside (*sic)*

2. The order of constitutional invalidity made in paragraph 1 above is hereby suspended for 12 months from the date of this order to enable the respondents to enact a law, in keeping with international best practices that ensures that sexually active children are accorded access to the sexual and reproductive health services and information required to mitigate the adverse health consequences of their sexual activities in accordance with the principle of the best interest of the child provided for in s81(3) of the constitution.

3. Each party shall bear its own costs.

Despite the notice of opposition indicating that the first to the third respondents are opposed, only one affidavit by one Aspect Maunganidze in his capacity as the permanent secretary in the first respondent's ministry was attached. He makes the following averments. That the Ministry respects the provisions in the Act which vary from the curative to preventative in nature. The Ministry has thus availed a package

of services to address SRHR. The constitution and the Act are aligned in the provision of these services hence the age of consent which is eighteen years. This is in recognition of the fact that children are not universally psychologically and mentally fit to make choices. Situations of children are different and there is no one size fits all approach. The assertion that the age of consent is a barrier for accessing health services is unfounded. The respondents pray that the application be dismissed.

In the answering affidavit, the applicant notes that essentially it is only the first respondent that is opposed to the application.

SUBMISSIONS

Applicant

The applicant submits as follows. That in constitutional interpretation, the text of the constitution is key- *Hewlett v Minister of Finance and anor,* 1981, ZLR 571. Section 46 of the constitution behoves a court, tribunal, forum or body to take into account various factors when interpreting it. Chief among these is the requirement to give full effect to the declaration of rights. The right sought to be asserted is found in s76. The purposive approach was outlined in the Canadian case of *R vs Big M Drug Mart Limited 18 DLR (4th).* The court is urged to also adopt a progressive interpretation as espoused by Lord Sankey in *Edwards v Attorney General Canada* , 1930 AC 124. Additionally, a generous interpretation should also be applied as espoused by Lord Wilberforce in *Minister of Home Affairs (Bermuda) vs Fisher,* 1979(3) ALL ER 121. It is clear from the wording of s76(1) of the constitution that the right to basic healthcare services including reproductive health care services is an entitlement of every citizen. This covers adolescents as well. This section specifically includes reproductive health services. While not defined in the constitution, the WHO definition is widely accepted. These are defined as a state of complete physical, mental and social wellbeing and not merely the absence of disease or infirmity, in all matters relating to the reproductive system and to its functions and processes. It should be noted that Zimbabwe is a party to international human rights instruments such as the International Covenant on Social and Cultural Rights (CESCR). Article 12 provides for the right to the highest attainable standard

of health. The court should also note the admission by the first respondent that adolescents are being provided with SRH services without parental or a guardian's consent on a case- by -case basis. Other jurisdictions such as South Africa in section 129 of the Children's Act number 38 of 2005 allow children over the age of 12 years or those that are sufficiently mature to consent to medical treatment and surgical procedures as well as access to contraceptives. A similar provision is found in section 232(2) of the Lesotho Children's Protection and Welfare Act 2011. Section 81 of the constitution clearly illustrates that in all things concerning a child, the best interests principle is paramount. While s86(2) of the Constitution provides for limitations to the rights protected under chapter 4, the impugned sections in the Act create a barrier to accessing SRHR. Such a limitation does not serve any of the interests specified in s86(2). All the respondents and in particular the first respondent are duty bearers. The state is expected as per s76(4) of the constitution to take measures for the progressive realisation of rights. The relief sought is in keeping with this obligation.

Respondents

The respondents call upon the court to exercise judicial restraint on matters that should essentially fall under the broad domain of the legislature- *Flether vs Peck (*1810). Children's rights are broad and the public has to be consulted- *R(Nicklison) vs. Ministry of Justice (2014 UKSC 38).* This matter involved the issue of assisted suicide under the Suicide Act of 1961. The majority of the bench deferred the issue to parliament on the basis that it was better suited to gather public views and make decisions on such a complex moral and social issue. Whilst s76(1) specifically mentions reproductive health rights, s81 on the rights of the child merely mentions healthcare services. The provisions of the constitution should not be read in isolation – *Shumba and ors vs Minister of Justice, Legal and Parliamentary Affairs and ors,* CCZ 4/18. It is clear that SRH were excluded from the rights of children because the legislature did not want them to be part of children's rights. The applicant's contention that the impugned rights also affect the right to life and to education cannot be sustained. Specifically with the right to education, the Education Act [Chapter *25:04*] provides that no one shall be excluded from school on the basis of

pregnancy. Reference to s56 of the constitution on equality and non-discrimination is vague. The impugned provision is one of the safe guards for children against sexual exploitation. By seeking that children seek SRH services with the consent of parents or guardians, the law is acting as protection of the children. The decision in *Mudzuru and anor vs Minister of Justice, Legal and Parliamentary Affairs and ors,* 2016 (2) ZLR 45 (CC) recognises the harm caused to children through early marriages and reinforces the notion that minors are incapable of giving informed consent to sexual relations. Should minor children then be able to access SRH services without parental or guardians consent? Section 60(3) of the constitution recognises the rights of parents and guardians to determine the upbringing of their children but not to the prejudice of their right under the constitution in relation to education, health, safety and welfare. If the impugned sections in the Act are declared unconstitutional, the rights provided in s60(3) of the constitution will be taken away. Regarding international conventions and practices and legislation from other countries, these have a persuasive effect. It is evident that different countries or regions impose different laws to regulate the rights of their citizen based on their economic, social, religious and moral beliefs.

THE LEGAL ISSUES

Whilst the applicant makes reference to other provisions of the constitution such as the right to life , education and equality, in my view the legal issue arising from the matter is the following:-

a. Is s35(1) as read with s35(2)(b) of the Public Health Act [Chapter *15:17*] in conflict with s76 and 81 of the constitution? In other words, is the impugned law inconsistent with the constitution?

This addresses the contention by the applicant that requirement for parental or a third party's consent constitutes a barrier to accessing SRHR and that this is an infringement of both the right to health care and and the best interests of the child principle.

THE LAW AND ITS APPLICATION

Section 2(1) of the constitution, states that it is the supreme law and any act or law that does not align with it is null and void as follows,

> "(1) This Constitution is the supreme law of Zimbabwe and any law, practice, custom or conduct inconsistent with it is invalid to the extent of the inconsistency."

The starting point is to outline the relevant sections of the Act and of the constitution. The impugned section reads as follows.

> **35 Consent of user**
>
> (1) For the purposes of this section "informed consent" means consent for the provision of a specified health service given by a person with legal capacity to do so and who has been informed as contemplated in section 34.
>
> (2) A health service shall not be provided to a user without the user's informed consent unless-
>
> (b) the user is unable to give informed consent and no person is mandated or authorised to give such consent, and the consent is given by the spouse or partner of the user or, in the absence of such spouse or partner, a **parent**, **grandparent**, **an adult child or a brother or a sister** of the user, in the specific order as listed;"

It is critical to also note that an adolescent, being a person aged between ten to nineteen years falls within the definition of a child. Hence, there will be emphasis on a child as well. The application is thus specific to those between ten and eighteen years.

The relevant provisions in the constitution read as follows.

76. Right to health care

(1) Every citizen and permanent resident of Zimbabwe has the right to have access to basic health-care services, including reproductive health-care services.

81. Rights of children

(1) Every child, that is to say every boy and girl under the age of eighteen years, has the right—

(a) to equal treatment before the law, including the right to be heard;

(b) ……..

(c) …….

(i) ……

(ii) …….

(d)……..

(e) to be protected from economic and sexual exploitation, from child labour, and from maltreatment, neglect or any form of abuse;

(f) to education, health care services, nutrition and shelter;

(g) ……

(h) ……..

(i) ……

(i) ……

(ii) ……..

(iii) ……

(2) A child's best interests are paramount in every matter concerning the child.

Any judge presiding over a matter that borders on law and morals is no doubt in an unenviable position. What the law states and what societal expectations are, may be at variance. My task is made less onerous in that there is, a clear path and an extensive body of knowledge on how courts should deal with matters relating to whether or not a law is in conflict with the constitution. To that end, I can do no better than cite extensively from the decision in *Kaweda vs. Minister of Justice, Legal and Parliamentary Affairs and ors,* CC-3-22 per MAKARAU JCC as follows;-

> "There is an expansive body of jurisprudence from this jurisdiction and beyond on the approach that a court must take when determining whether a statute or other law is in conflict with the Constitution. One begins with an interpretation of the relevant provisions of the Constitution. The purpose of interpreting the Constitution first is to set the framework, the backdrop, or the yardstick against which the impugned law will then be examined or measured. One starts with a discernment of the law. (See *Zimbabwe Township Developers (Pvt) Ltd v Lous' Shoes (Pvt) Ltd* 1983 (2) ZLR 376 (SC) at 383 F; and *Democratic Assembly for Restoration and Empowerment & Ors v Suanyama* CCZ 9/18).
>
> In interpreting the constitutional provisions, the ordinary rules of interpretation of statutes apply. The Constitution is but a statute. It is however settled that in interpreting constitutional provisions, the preferred construction "is one which serves the interest of the Constitution and best carries out its objects and promotes its purpose". (See *Rattigan and Others v The Chief Immigration Officer and Others* 1994(2) ZLR 54. See also *Smythe v Ushewokunze and Another* 1997(2) ZLR 544(S)). In particular, when interpreting provisions that guarantee fundamental rights, the widest possible interpretation is adopted to give each right its fullest measure or scope.
>
> After interpreting the appropriate provisions of the Constitution, one then presumes that the impugned law is constitutionally valid. The presumption of constitutional validity serves firstly to place the onus on whoever is alleging invalidity to prove such invalidity and, secondly and, equally important, to guide the court in interpreting the impugned law in favour of validity where the piece of legislation is capable of two meanings. The presumption holds that where a piece of legislation is capable of two meanings, one falling within and the other falling outside the provisions of the Constitution, the court must perforce uphold the one that falls within.
>
> The presumption in favour of constitutionality is entrenched in our law.
>
> As the next and final logical step, the Court must then examine the effect of the impugned law on the fundamental right or freedom in question. If the effect of the impugned law is to abridge a fundamental right or freedom or is inconsistent with the provisions of the Constitution providing for the right or freedom, the object or subject matter of the impugned law will be less important or irrelevant. (See *In re Mhunhumeso* 1994 (1) ZLR 49 (S)).
>
> If the court finds the impugned law to infringe upon a fundamental right or freedom or to be inconsistent with the provisions of the Constitution on a fundamental right or freedom, the court must proceed to determine whether the infringement or inconsistency is permissible in terms of s 86 (2) of the Constitution.
>
> The step- by- step approach that I have laid out above is to be found in a line of cases that includes *Zimbabwe Township Developers (Pvt) Ltd v Lous' Shoes (supra); In re Mhunumeso (Supra) James v Zimbabwe Electoral Commission and Others* 2013 (2) ZLR 659 (CC) and *Democratic Assembly for Restoration and Empowerment & Ors v Saunyama (supra)*."

Turning now to the relevant constitutional provisions. Section 76(1) specifically includes the right to reproductive health care services and this is not accidental. It is deliberate. As per the *Rattigan* case cited in the *Kaweda* matter (supra) on rules of constitutional construction,

> "This Court has on several occasions in the past pronounced upon the proper approach to constitutional construction embodying fundamental rights and protections. What is to be avoided is the imparting of a narrow, artificial, rigid and pedantic interpretation; to be preferred is one which serves the interest of the Constitution and best carries out its objects and promotes its purpose. All relevant provisions are to be considered as a whole and, where rights and freedoms are conferred on persons, derogations therefrom, as far as the language permits, should be narrowly or strictly construed. See Minister of Home Affairs and Others v Dabengwa and Another 1982 (4) SA 301 (ZS) at 306E-H (1982 (1) ZLR 236 (SC) at 243G-244A); Bull v Minister of Home Affairs 1986 (3) SA 870 (ZH and ZS) at 880J-881D (1986 (1) ZLR 202 (SC) at 210E-21 IC); Nkomo and Another v Attorney-General, Zimbabwe, and Others 1994 (3) SA 34 (ZS) (1994 (1) SACR 302) at 41B-C (SA) and 309e-f (SACR). A recent reminder that Courts cannot allow a Constitution to be 'a lifeless museum piece' but must continue to breathe life into it from time to time when opportune

to do so, was graphically expressed by Aguda JA in Dow v Attorney-General [1992] LRC (Const) 623 (Botswana CA) at 668f-h:

> '. . . (T)he overriding principle must be an adherence to the general picture presented by the Constitution into which each individual provision must fit in order to maintain in essential details the picture which the framers could have painted had they been faced with circumstances of today. To hold otherwise would be to stultify the living Constitution in its growth. It seems to me that a stultification of the Constitution must be prevented if this is possible without doing extreme violence to the language of the Constitution. I conceive it that the primary duty of the Judges is to make the Constitution grow and develop in order to meet the just demands and aspirations of an ever-developing society which is part of the wider and larger human society governed by some acceptable concepts of human dignity.'

See, too, Hunter et al v Southam Inc (1984) 9 CRR 355 (SCC) at 364 and Government of the Republic of Namibia and Another v Cultura 2000 and Another 1994 (1) SA 407 (NmS) at 418F-G".

Breathing life into the 2013 constitution in my view means that the right to reproductive health as enshrined applies to everyone including adolescents as rightly contended by the applicant.

The courts have also interpreted the notion of best interests of a child, who is described as any person below the age of eighteen years in relation to custody and access- see *In re Sibanda,* HH-207-18.

In instances where the definition does not cover issues outside the context of guardianship, custody and access, subregional, regional and internation instruments come in. That this is so was recognised by HUNGWE J (as he then was) in *Dangarembizi vs, Hunda,* HH-447-18 as follows,

> "There are a number of legal instruments that specifically provide for the best interest of the child as a legal norm. This principle is a well-established principle of the international, regional and national normative framework. The principle is also entrenched and provided for in United Nations human rights legal instruments; in addition, the African Union's African Charter on Human and Peoples' Rights (the African Charter) and the African Charter on the Rights and Welfare of the Child (ACRWC) provide for it, and national constitutions in their children's rights provisions make reference to it…….. There are a number of legal instruments that specifically provide for the best interest of the child as a legal norm. This principle is a well-established principle of the international, regional and national normative framework. The principle is also entrenched and provided for in United Nations human rights legal instruments; in addition, the African Union's African Charter on Human and Peoples' Rights (the African Charter) and the African Charter on the Rights and Welfare of the Child (ACRWC) provide for it, and national constitutions in their children's rights provisions make reference to it. Giving effect to the principle in question buttresses Zimbabwe's obligations under international and regional instruments. In the event that domestic law, including the country's Constitution does not adequately protect the rights of the child, the safeguards provided by the Convention on the Rights of the Child (CRC) should be sought. Happily, our new Constitution does not suffer from such a defect".

In terms of s81(1) of the constitution, the right to be heard is a fundamental one for a child. The right to health -care services is specifically established in s81(1)(f). In *Chihava & Ors* v *The Provincial Magistrate N.O* CCZ6/15 the court held as follows:

> "Thus, while a constitution is to be interpreted in accordance with the rules relating to statutes generally, the authorities suggest that a court called upon to interpret such a constitution should give a generous and purposive construction to its provisions, particularly the entrenched fundamental rights and freedoms. It appears to me that in this process the purpose of the provision in question is to be given particular attention."

In his book *'Principles of Legal Interpretation - Statutes, Contracts and Wills'* 1st Ed. At page 57, E A Kellaway echoes this statement as follows:

> "The dominating Roman-Dutch law principle is that an interpretation which creates an absurdity is not acceptable (that is *'interpretatio quae parit absurdum, not est admittenda)"* (See among other authorities, Exparte Fourie 1962 (3) SA 614 (O); *S v Nyathi* 1978 (2) SA 20 (B) and *Canca V Mount Free Municipality* 1984 (2) SA 870 (TK) 833)"

This purposive interpretation in my view bears credence to the fact that the rights of adolescents to health generally and to SHR specifically are covered in the constitution. As per the *Kaweda* case, (*supra*) I must still assume that the impugned provisions are constitutionally valid. The applicant both in the founding affidavit and submissions have provided guidance to the court on how to interpret the impugned provisions. They have attached a report some of whose findings have been outlined in the introduction. They have shown how the issue of parental and third -party consent constitutes a barrier to access to SRH. Further, they have illustrated how these impact on the lives of adolescents especially girls. The further affidavit attached being an acknowledgment by a representative of the first respondent that there is a grave problem of teenage pregnancies is illustrative. The challenge of teenage pregnancies continues unabated . The applicant's case is further supported by the admission by the first respondent that despite the requirement of parental or guardian's consent, the Ministry is as a matter of fact assessing each case on its merits. This relates to the provision of SRH and services to adolescents.

I accordingly conclude that the impugned provision in so far as it affects access to SRH services by adolescents through the requirement of informed consent through a parent or other third party is unconstitutional. The right in s76(1) remains unqualified meaning it applies to everyone. The best interests of the child widely interpreted as backed by the international human rights framework supports the inclusion of children in decisions that affect them. This is backed by s81 of the constitution on the right to be heard by a child. How then can this be exercised in relation to SRH if it is qualified by reference to a third party who may or may not be willing to give consent?

I now turn to s86(2) of the constitution that reads,

86. **Limitations of rights and freedoms**

(1)……

(2) The fundamental rights and freedoms set out in this chapter may be limited only in terms of a law of general application and to the extent that the limitation is fair, reasonable, necessary and justifiable in a democratic society based on openness, justice, human dignity, equality and freedom, taking into account all relevant factors including:

(a) the nature of the right or freedom concerned;
(b) the purpose of the limitation, in particular whether it is necessary in the interests of defence, public safety, public order, public morality, public health, regional or town planning or the general public interest.
(c) The nature or the extent of the limitation;
(d) The need to ensure that the enjoyment of rights and freedoms by any person does not prejudice the rights and freedoms of others,
(e) The relationship between the limitation and its purpose, in particular whether it imposes greater restrictions on the right or freedom concerned than are necessary to achieve its purpose and
(f) Whether there are any less restrictive means of achieving the purpose of the limitation.

Parental or guardian's consent is a critical feature under Zimbabwe's laws. The Children's Act [Chapter *5:*07] for instance is underpinned by the notion that although children are holders of rights, they cannot exercise these on their own. The same applies to the Guardianship of Minors Act [Chapter *5:08*] in dealing with issues of custody and access. The Maintenance Act [Chapter *5:09*] in as much as it recognises the fundamental right of a child to maintenance does not provide that the child who is under 18 can approach the court on their own and apply for maintenance. The Birth and Death Registration Act [Chapter *5:02*] has no provision for a child obtaining a birth certificate on their own. This must be done on their behalf by a parent. In other words, there is a recognised vulnerability of the child and that someone must exercise these rights on their behalf. A minor cannot obtain a passport on their own. This is what is termed the protectionist treatment of children, that views them as vulnerable beings. The respondents have posed an important question that directly addresses s86(2) on why there should be an exception for SRHR?

In the *Mudzuru* case (supra) the court relied extensively on the UNICEF report that it found it worth to set out infringements and consequences of child marriages as follows,

4.8. Health Costs

Child marriage reinforces the incidence of infectious diseases, malnutrition, high child mortality rates, low life expectancy for women, and an inter-generational cycle of girl-child abuse. Pregnancy-related death is a leading cause of death for girls between 15 and 19 years of age. The dangers of early marriage affect not only the girl child but the child born to

her as well. Premature birth, low growth rate and poor mental and physical growth are some characteristics of babies born to young mothers.

The real costs associated with women's health and infant mortality are enormous. Child marriage can have devastating consequences on the sexual and reproductive health of girls: specifically increasing the risk of maternal mortality and morbidity and contracting sexually transmitted diseases, particularly HIV/AIDS. The risk of contracting STI's and HIV rises and married girls are unable to negotiate safe sex and are more likely to be married to older men with more sexual experience who are more likely than single men to be HIV positive.

Young girls particularly those below 15 years of age, face serious reproductive health hazards sometimes losing their lives as a result of early pregnancies. Those under the age of 15 are five times as likely to die as women in their twenties. The main causes are haemorrhaging, sepsis, pre-eclampsia/eclampsia and obstructed labour. When a young mother's vagina, bladder or rectum tears during child birth, it can cause urine or faeces leakage known as obstetric fistula. This can happen when a young woman with underdeveloped physiology gives birth.

In addition to their lack of power in relation to their husbands or in-laws, girls are further exposed to **sexual and reproductive health problems because of their lack of knowledge, information and access to sexual and reproductive health services, in particular family planning, ante-natal, obstetrics and post-natal care**. *(my emphasis).*

The realm of children's rights within which adolescents fall is filled with tension. This is clearly illustrated when regard is had to child rights theories. Laurance Holgate, authored an article titled, *" Three concepts of children's constitutional rights: Reflections on the enjoyment theory",* (Journal of constitutional law) Volume 2:1, December 1999. The author @page 4 postulates as follows, with respect to the developments under United States laws,

" In the thirty- year history of its rulings on the constitutional rights of children, the Supreme Court appears to have struggled with the following dilemma; either children are to be regarded as persons with fundamental rights and the state must respect, or they are to be regarded as *human beings,* who are always in some form of custody. If children are persons with fundamental rights, then how can the Court justify the different treatment that children receive under the Constitution? That is, if both children and adults are, "persons", then shouldn't children and adults have the same set of Constitutional rights? On the other hand, if children are categorised as humans who are always in some form of custody, then why should they be accorded any constitutional rights at all? ……There are three possible solutions to this dilemma. Each solution proposes an interpretation that attempts to accommodate the fact that children are under the control of either their parents or the State. The solutions are these: (1) children's constitutional rights are rights of a scope that is more *limited* than the scope of adult constitutional rights of the same name (e.g, the right to freedom of speech); (2) children's constitutional rights are rights that can be *outweighed* by important familial or State interests which do not outweigh adult rights of the same name; (3) children's constitutional rights are *rights-in-trust*, that is rights which children possess but which, in some cases, they are justifiably prevented from enjoying."

He draws a distinction between situations where a child is clearly incapable of exercising a right and one where they can do so. It seems the age and maturity of the child is the determining factor.

In Kruger's, '*The theory of children's rights- An overview',* there is an extensive discussion on the rights of the child. For instance under the will theory of rights, there is a refusal to award rights to children merely because they do not have

the capacity for reasoned decision making. Under the paternalism of Hobbes in the 17th century, children were seen as wards of their fathers and therefore possessed no natural rights. According to John Locke, children are in a temporary state of ignorance and irrationality though this may ultimately make way for maturity. In the 19th century, John Mills' theory was that the doctrine of ultimate value of personal choice does not extend to children. It is critical to note that the real of children's rights has not been static. For instance under the Rawls theory of justice, everyone should be permitted to act according to personal conception of her or his own's best interests but not at the expense of others. What this shows is that there is no consensus on the nature of these rights.

The applicant has made reference to the South African and Lesotho legal positions as already outlined. In South Africa, the legal position is as per s129 of the Children's Act Number 38/2005.

> **129. Consent to medical treatment and surgical operation** .
>
> (2) A child may consent to his or her own medical treatment or to the medical treatment of his or her child if-
>
> (a) the child is over the age of 12 years; and
>
> (b) the child is of sufficient maturity and has the mental capacity to understand the benefits, risks, social and other implications of the treatment.

In Lesotho, the position is as follows as per the Children's Protection and Welfare Act of 2011.

> **232. Consent to medical treatment or surgical operation**
>
> (2) A child may consent to medical treatment provided the child is—
>
> (a) at least twelve years of age; and
>
> (b) of sufficient maturity and has the mental capacity to understand the benefits, risks, social and other implications of the treatment or operation.

The position in South Africa and Lesotho is therefore similar.

In Namibia, the position is governed by s220 of the Child Care and Protection Act [Number 3 of 2015] as follows:-

> **220. Consent to medical intervention and surgical operation**
>
> (1)Despite any law to the contrary
>
> -(a)a child may consent to a medical intervention in respect of himself or herself if

> -(i)the child is 14 years of age or older; and(ii)a medical practitioner concerned is satisfied that the child is of sufficient maturity and has the mental capacity to understand the benefits, risks and implications of the medical intervention;

This is read to include SRHR services.

It seems across the world; there is no one size fits all approach. In the Netherlands, under the Medical Treatment Contracts Act, there is a three- tiered approach. For those aged sixteen years and above, they have full legal capacity to consent to SRH services without any parental involvement or notifications. For those aged between twelve and fifteen years, there is double consent where the minor and a parent/guardian must agree. There is also what is called the safety-valve even for those aged between twelve and fifteen where access to abortion without parental consent can occur if the doctor deems the minor to be competent and that involving the parent will be harmful of dangerous. In the United Kingdom, there is what is termed the Gillick and Fraser competence under the common law. These guidelines are to the effect that persons aged below sixteen years of age can consent to their SRH treatment if they are deemed to have sufficient intelligence and understanding to full appreciate what they are going through. Under the Fraser guidelines, there is a specific- criteria that doctors must meet to provide SRH advice to those under sixteen years without parental consent. These include that the young person understands the advice; that they cannot be persuaded to inform their parents; that the young person is likely to continue having sexual intercourse with or without contraceptive treatment; that unless they receive treatment, their physical or mental health is likely to suffer and that it is in their own best interests. In Scotland, under the Age of Legal Capacity (Scotland) Act of 1991, which mirrors the Gillick principle, a stronger statutory right is provided to those under sixteen years to consent if they understand the nature of the treatment.

Under the United Nations framework, Articles 5 and 24 of the Convention on the Rights of the Child (CRC) provides as follows:-

Article 5

States Parties shall respect the responsibilities, rights and duties of parents or, where applicable, the members of the extended family or community as provided for by local custom, legal guardians or other persons legally responsible for the child, to provide, in a manner consistent with the **evolving capacities** of the child, appropriate direction and guidance in the exercise by the child of the rights recognized in the present Convention.

Article 24

States Parties recognize the right of the child to the enjoyment of the highest attainable standard of health and to facilities for the treatment of illness and rehabilitation of health. States Parties shall strive to ensure that no child is deprived of his or her right of access to such health care services.

The African Charter on the Rights and Welfare of the Child while emphasising that children have rights and responsibilities makes a general provision in Article 14(2) (b) that states must ensure the provision of necessary medical assistance and health care to

The analysis of the positions in different jurisdictions , the CRC and the ACRWC brings to the fore the concept of evolving capacities of children. Such is not alien in our law. In *Sogolani vs. The Minister of Primary and Secondary education and ors,* CCZ-20-20, the court per MALABA C.J postulated as follows,

> "The principle of evolving capacities of the child by which a parent or guardian is required by Article 14(2) of the *CRC* to be guided when providing appropriate direction and guidance to the child in the exercise of his or her right to freedom of religion was discussed in a document titled "*Evolving Capacities*" prepared for UNICEF and SAVE THE CHILDREN. The author of the document said:
>
> > "The concept of evolving capacities is central to the balance embodied in the Convention between recognising children as active agents in their own lives, entitled to be listened to, respected and granted increasing autonomy in the exercise of rights, while also being entitled to protection in accordance with their relative immaturity and youth. This concept provides the basis for an appropriate respect for children's agency or their capacity to act and make decisions without exposing them prematurely to the full responsibilities normally associated with adulthood. As children grow up, they have evolving capacities and a growing understanding and maturity."

More poignantly, the *Sogolani* case reveals the tension between the parental rights and the notion of children being rights holders. The court also had occasion to deal with the best interests of the child principle as follows,

> > "It is clear that the overriding principle that the best interests of the child shall be a primary consideration in all actions concerning the child has become universally acceptable. It is expressly provided for under s 81(2) of the Constitution, which provides that "a child's best interests are paramount in every matter concerning the child". A parent or guardian is bound by the Constitution when performing the primary responsibility of bringing up a child.
>
> > There is, however, no fixed standard of what will be in the best interests of the individual child. The law does not start from a prior assumption about what is best for any individual child. It looks at the child and weighs a number of factors in the balance, depending on the circumstances of the case. See *In Re J (Child Returned Abroad: Convention Rights)* [2006] 1 AC 80 paras 37-39".

Having outlined the above, in my view, the impugned sections are an infringement on the rights of adolescents to their SRH and to the best interests principle. I now turn to s86(2) of the constitution. I find guidance in the *Sogolani* case (*supra)* on the interpretation of that section as follows,

> > "Having come to the conclusion that the compulsory recitation of the pledge in schools infringes the rights guaranteed under s 60 of the Constitution, the next inquiry is whether the pledge can be saved under s 86(2) of the Constitution or must be declared constitutionally invalid.

Section 86(3) of the Constitution makes provision for fundamental human rights which are non-derogable. The right to freedom of religion is not one of them. It is not unlimited. The justification for limitation of fundamental human rights and freedoms is the principle that they must be reasonably exercised and with due regard for the rights and freedoms of other persons.

The principle enshrined in s 86(1) of the Constitution recognises the fact that, like all the fundamental rights, freedom of religion guaranteed under s 60 has as its point of departure the view of a human being in the Constitution as a responsible personality, developing freely within the social community. It can be restricted by the Legislature by a law of general application with constitutional anchors and sufficient safeguards for the rule of law when community goods are endangered.

Section 86(2) of the Constitution contains requirements, the contents of which include compelling public interests that would justify certain restrictive State actions and limit the exercise of the right to freedom of religion. The State can limit the exercise of the right to freedom of religion only to the extent and in the manner prescribed under s 86(2) of the Constitution.

An examination of the contents of the requirements of acceptable limitation of a derogable fundamental right or freedom shows that the object of s 86(2) of the Constitution is to ensure that the essence of the fundamental right or freedom is preserved. The primary constitutional duty on the State is to protect and promote fundamental human rights and freedoms enshrined in *Chapter 4*. The power to limit the exercise of derogable fundamental human rights and freedoms is an exception to the primary duty to respect, protect, promote and fulfil fundamental human rights and freedoms. It is for this reason that limitations to the exercise of fundamental human rights and freedoms must be construed strictly and narrowly, whilst fundamental rights and freedoms must be given broad and generous interpretation".

Further, that,

"Section 86(2) sets the minimum requirement for the limitation of a fundamental right. A proper reading of the section points to the fact that only a law of general application may limit a right enshrined in *Chapter 4* of the Constitution. The limitation section is premised upon the fundamental tenet of the rule of law, which reinforces the idea that public authority may only be exercised where the law clearly provides for it.

Currie and De Waal in "*The Bill of Rights Handbook*" (6 ed, Juta & Co (Pty) Ltd 2013) at p 155 describe the concept behind the requirement that the limitation must be in terms of a law of general application as follows:

> "The 'law of general application' requirement is the expression of a basic principle of liberal political philosophy and of constitutional law known as the rule of law. There are two components to this principle. The first is that the power of the government derives from the law. The government must have lawful authority for its actions, otherwise it will not be a lawful government but will be despotism or tyranny."

In *Chimakure and Ors* v *The Attorney-General of Zimbabwe* 2013 (2) ZLR 466 (S), the Court fortified the position that the rights enshrined in *Chapter 4* can only be limited in terms of the law. The Court held as follows at pp 495F-496A:

> "It is a fundamental principle of constitutional law that any restriction which hinders the enjoyment of a fundamental right must be introduced by a legal provision. The grounds for the justification of the restriction must be found in the law by which it is imposed. Fundamental rights and freedoms and other constitutional values are protected by the fundamental law which is the supreme law of the land. Restrictions imposed on them must be consistent with the fundamental law otherwise they are void.
>
> The requirement that the restriction on the exercise of the right to freedom of expression must be contained in law is expressive of and consistent with the principle of the rule of law. The principle is to the effect that every governmental action which adversely affects the legal situation of persons in a free and democratic society must be justifiable by reference to an existing law."

The *dictum* of NGCOBO J in *Affordable Medicines Trust and Others* v *Minister of Health and Others* 2006 (3) SA 247 (CC) is apposite. It was held as follows at para 49:

> "The exercise of public power must therefore comply with the Constitution, which is the supreme law, and the doctrine of legality, which is part of that law. The doctrine of legality, which is an incident of the rule of law, is one of the constitutional controls through which the exercise of public power is regulated by the Constitution."

While interpreting the requirement of s 1 of the Canadian Charter of Rights and Freedoms, to the effect that a limit on the exercise of freedom of expression must be "prescribed by law", BROWNRIDGE JA in *Regina* v *Therens* [1985] 13 CRR 193 at p 216 held as follows:

> "The requirement that the limit be prescribed by law is chiefly concerned with the distinction between a limit imposed by law and one that is arbitrary. The limit will be prescribed by law within the meaning of s 1 if it is expressly provided for by statute or regulation or results by necessary implication from the terms of the statute or regulation or from its operating requirements. The limit may also result from the application of a common law rule."

See also *R* v *Thomsen*, 1988 CanLII 73 (SCC), [1988] 1 S.C.R. 640 at pp 650-51.

In *McKinney* v *University of Guelph*, 1990 CanLII 60 (SCC), [1990] 3 S.C.R. 229, the Supreme Court of Canada per WILSON J remarked as follows at p 386:

> "Section 1 … serves the purpose of permitting limits to be imposed on constitutional rights when the demands of a free and democratic society require them. These limits must, however, be expressed through the rule of law. The definition of law for such purposes must necessarily be narrow. Only those limits on guaranteed rights which have survived the rigours of the law-making process are effective." (the underlining is for emphasis)

It cannot be gainsaid that a limitation of a fundamental right enshrined in the Constitution must be in terms of a constitutional provision, a statute and/or its subordinate legislation. This is because the limitation of a right must, of necessity, be as a result of a meticulous and extensive legislative process. The limits should be interpreted narrowly, thereby respecting the importance of the fundamental right or freedom sought to be protected and enforced".

My understanding is that as courts, we must err on the side of protecting fundamental rights as set out in the constitution. There must be of necessity a balancing act between the right to SRH and acting in children's best interests and that of parental rights and obligations.

In *Democratic Assembly for Restoration and Empowerment and ors. Vs. Saunyama N.O and others,* per Makarau JCC, on the saving under s86(2);

> "Before I proceed to consider whether or not s 27 of POSA can be saved under s 86 (2) of the Constitution, I will briefly discuss the relationship between the general approach that a court takes in considering the constitutional validity of a challenged piece of legislation and the specific test that the court must apply as directed in s 86 (2) of the Constitution. I am detained in this regard as it appears that there may be some confusion as to whether or not the general approach that was laid out in case law prior to the enactment of the Constitution remains applicable in light of the express provisions of s 86 (2) of the Constitution.
>
> Counsel for the applicants argues that s 86 (2) has codified the approach that the court must take in construing a limitation and suggests that the court should look no further. I do not agree.
>
> I am inclined towards the broad view expressed by PATEL JA in *James v Zimbabwe Electoral Commission and Others* 2013 (2) ZLR 659 (CC), wherein at p 666E he held that:
>
> "Section 86 (2) of the Constitution is essentially a restatement of the criteria for permissible derogation from constitutional rights as enunciated by the Supreme Court in *Nyambirai v National Social Security Authority & Another* 1995 (2) ZLR 1 (S))".
>
> In *Nyambirai v NSSA* (*supra*), the court, relying on the Canadian case of *R v Oakes* (1986) 19 CRR 308, had held that:
>
> "In effect the court will consider three criteria in determining whether or not a limitation is permissible in the sense of not being shown to be arbitrary or excessive. It will ask itself whether:
>
> (i) The legislative objective is sufficiently important to justify limiting a fundamental right;
> (ii) The measures designed to meet the legislative objective are rationally connected to it; and
> (iii) The means used impair the right or freedom are no more than is necessary to accomplish the objective."
>
> I read the view by PATEL JA as holding that the provisions of s 86(2) of the Constitution and the general approach to establishing permissible limitations to constitutional rights are complementary and not mutually exclusive and that both are applicable.
>
> Thus, the general approach that has been discussed in cases such as *In re Mhunumeso and Others* 1994 (1) ZLR 49 (S), *Nyambirai v NSSA (supra)*, *Retrofit (Private) Limited v PTC and Anor* 1995(2) ZLR 199 and *Chimakure and Others v AG* 2013 (2) ZLR 466 (S) which were decided before the promulgation of the Constitution remains valid as providing general guiding principles while s 86 (2) sets out in detail the factors that a court must take into account in determining whether or not a limitation of a fundamental right is constitutional".

Further that,

> "Clearly, the law directs the court to test the infringing law under four specific heads. These are whether such a law is fair, reasonable, necessary and justifiable in a democratic society based on openness, justice, human dignity, equality and freedom. In testing the infringing law against these specific yardsticks, the court is enjoined to take into account **all relevant factors** including the factors spelt out in the section.
>
> I would therefore venture to suggest that s 86 (2) simply gives the court an array of some of the factors to take into account before it comes up with what is essentially a value judgment. The list given is not exhaustive as the law enjoins the court to take into account all relevant factors including the ones that it spells out. Thus, the court must be holistic both in its approach and in its finding".

Taking a cue from the *Democratic Assembly for Restoration and Empowerment* matter (supra), I will proceed to assess the limitation in s86(2).

ISs35(1) AS READ WITH s35(2)(b) OF THE PUBLIC HEALTH ACT REASONABLE, NECESSARY AND JUSTIFIABLE IN A DEMOCRATIC SOCIETY BASED ON OPENNESS, JUSTICE, HUMAN DIGNITY, EQUALITY AND FREEDOM?

The definition of informed consent in respect of adolescents for access to SRH services shows the never- ending tension between the rights especially of children and parents. I note that the applicant has made reference to the position in South Africa and Lesotho. It is critical in my view to note that in these and other jurisdictions, there is no one-size fits all approach. In South Africa and Lesotho, the age of twelve is conditional upon the sufficient maturity and mental capacity of the child aged above twelve and above in understanding the nature of their SRHR. This means that even if a child is aged twelve and above, once it is shown that they are immature, they will not benefit from this law. On one hand, the laws of Zimbabwe are underpinned by protectionism, the very notion that a child is vulnerable and they need support from a parent or a guardian. In the *Kaweda* case, (*supra)* children are not supposed to even be having sexual intercourse if aged below eighteen and that is why sexual intercourse with a child attracts criminal consequences. In the *Mudzuru* matter, a person below the age of eighteen as espoused further in the Marriages Act [Chapter 5:17] is not supposed to be married. Anyone including a

parent who participates in a child marriage faces severe sanctions. For an adolescent accessing SHR services, it means someone is violating them or someone will violate them and this constitutes a criminal offence. While not discounting the devastating consequences of child pregnancies and the prevalence in Zimbabwe, in my view, the law limiting access to adolescents to SRHR services without a parent or a third party's consent is fair and reasonable. It is meant to protect children not only from sexual exploitation but also ensuring that the perpetrators are brought to book. The limitation is justifiable for the same reasons. We cannot on one hand protect adolescents through enacting laws that prohibit exploitation and on one hand allow access to SRH services minus the involvement of a parent or other third party. Allowing such would essentially turn the adolescent into an adult as was the case in the then Marriages Act [Chapter 5:11] under which a child aged between sixteen and seventeen who got married would automatically become an adult. The limitation is thus fair, reasonable, necessary and justifiable.

I note that the doctrine of evolving capacities of children is gaining traction. Even the CRC which is the universal bill of rights and to which Zimbabwe is a party recognises this concept. A child aged sixteen might be better able mentally and emotionally to receive SHR services than a child aged ten years and yet they all fall into the definition of an adolescent. The applicant has not pleaded evolving capacities yet seeks to rely on foreign jurisdictions that have made use of this concept in accordance with what is prevailing in their own countries. The applicant seeks a blanket declaration of constitutional invalidity meaning that if granted, a child aged ten or eleven can get access to SRH unaccompanied. Granted, I may seem to be wading into the Hart/Fuller debate on law and morals. Suffice to state that even

when consulting a child in exercising the right to be heard, there is no one -size fits all. A more mature child may be better able to articulate their views than a younger one. I agree with the respondents that the court should exercise judicial restraint and allow parliament to make adequate consultations as guided by the evolving capacities doctrine.

The applicant has not sought any costs and in any event it is trite that such should rarely be granted in constitutional cases.

DISPOSITION

No doubt, the applicant has raised a critical issue with respect to reproductive health care and rights by adolescents. This is more so against the alarming rates of under-age pregnancies. While, the impugned sections breach s76(1) and 81 of the constitution, the limitation is saved by s86(2).

Accordingly, it is ordered as follows:-

1. The application be and is hereby dismissed.
2. There shall be no order as to costs.

Legal Resources Foundation, applicant's legal practitioners

Civil-Division of the Attorney-General's office, respondents" legal practitioners